

TRAUMATISED/ABUSED PATIENT

Give urgent attention to the traumatised/abused patient with any of:

- Injuries needing attention ↗ 18
- Suicidal thoughts or behaviour ↗ 83
- Recent rape or sexual assault

Management of recent rape/sexual assault:

- Arrange same day doctor assessment, ideally at a designated facility for management of rape and sexual assault. Complete required forms and registers. If rape victim pregnant, refer.
- If severe vaginal or anal bleeding, abdominal pain, multiple injuries or history of the use of a foreign object, refer urgently.
- Prevent HIV and hepatitis ↗ 108.
- Prevent STIs: give single dose each of **ceftriaxone** 250mg IM¹, **azithromycin** 1g orally and **metronidazole**² 2g orally. If severe penicillin allergy³, omit ceftriaxone and increase **azithromycin** dose to 2g orally.
- Prevent pregnancy: do pregnancy test. If pregnant ↗ 159. If not pregnant, not on reliable contraception and ≤ 5 days since rape, give emergency contraception:
 - Give single dose **levonorgestrel** 1.5mg orally.
 - If patient > 80kg, BMI⁴ ≥ 30, or on efavirenz, rifampicin, phenytoin or carbamazepine, increase dose of **levonorgestrel** to 3mg or offer **IUD** instead.
 - Give **metoclopramide** 10mg 8 hourly as needed for nausea/vomiting. If patient vomits < 2 hours after taking levonorgestrel, repeat dose or offer **IUD** instead.
- Prevent tetanus: if open wound and not immunised in last 5 years, give **tetanus toxoid** 0.5mL IM within 48 hours of injury.
- Also assess and support the patient as below.

Assess the traumatised/abused patient

Assess	When to assess	Note
Symptoms	Every visit	Manage symptoms as on symptom pages. Ask about genital symptoms even if no recent rape or sexual assault ↗ 49.
Family planning	Every visit	Offer to start longterm contraceptive ↗ 154. If sexual assault and normal menstruation has not occurred within 4 weeks, repeat pregnancy test. If pregnant ↗ 159.
Mental health	Every visit	• If stress or anxiety ↗ 86. • In the past month, has patient: 1) felt down, depressed, hopeless or 2) felt little interest or pleasure in doing things? If yes to either ↗ 143. • If patient has ever had an experience so horrible that s/he has had ≥ 3 of the following for > 1 month: 1) Nightmares or involuntary thoughts/flashbacks 2) Avoided certain situations/people 3) Been constantly on guard, watchful or easily startled 4) Felt numb or detached from other people, activities or surroundings: post-traumatic stress disorder likely, refer.
Alcohol/drug use	Every visit	In the past year, has patient: 1) drunk ≥ 4 drinks ⁵ /session, 2) used illegal drugs or 3) misused prescription or over-the-counter medications? If yes to any ↗ 142.
Social	Every visit	If immediate risk of being harmed and in need of shelter, refer/discuss with social worker same day.
HIV	First visit	Test for HIV ↗ 110.
Syphilis (if sexual assault)	First visit	If positive ↗ 53.

Advise the traumatised/abused patient

- Find a quiet place to talk. Comfort patient, remind him/her that you are there to help. Reassure that s/he is safe and all information is confidential. Allow a trusted friend/relative to stay close.
- Be patient, listen attentively and avoid pressuring the patient. Clearly record patient's story in his/her own words. Include nature of assault and, if possible, identity of the perpetrator.
- Ask if patient has specific needs/concerns and link with support structures. Refer to available trauma counsellor/psychiatric nurse/psychologist/social worker/helpline ↗ 178.
- Refer to police Victim Empowerment office or family violence NGOs for assistance.
- Encourage patient to file a J88 form and to report case to police. Encourage patient to apply for protection order at local magistrate's court. Respect patient's wishes if s/he declines to do so.

If rape/sexual assault, review within 3 days ↗ 109. Offer to review the traumatised/abused patient who has not been sexually assaulted in 1 month.

¹For ceftriaxone 250mg IM injection: dissolve 250mg in 0.9mL **lidocaine 1%** without epinephrine (adrenaline). ²Advise no alcohol until 24 hours after last dose of metronidazole. ³History of angioedema, anaphylaxis or urticaria.

⁴BMI = weight (kg) ÷ height (m) ÷ height (m). ⁵One drink is 1 tot of spirits, or 1 small glass (125mL) of wine or 1 can/bottle (330mL) of beer.